

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

***Congress Attendance Report — American Academy of Neurology Annual Meeting (AAN) 2026***

Meeting period: April 2026 | Author: Priya Menon

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

***Conversation — Priya Raghavan, MD (North Star Neurology)***

I met the physician at the Thursday registration desk, and the discussion concerned telehealth logistics only. Two operational points were raised: (1) video f/u works well for stable pts, although seizure-diary review is awkward without a shared screen, and (2) out-of-state pts have raised licensure questions that the group is still working through. I confirmed that no action from my side was required and closed the conversation there.

***Conversation — Robert Enloe, MD (High Desert Neurology)***

A short conversation followed the Thursday afternoon session, again on administrative burden rather than anything clinical. The substance was as follows: prior-authorization paperwork now consumes a considerable share of one staff member's week, appeals are frequently resubmitted for formatting reasons alone, and no product or payer was named at any point. I offered no materials, as the topic sits outside my remit.

***Conversation — Grace Lindqvist, MD (Riverstone Medical Group)***

I was approached during the Friday poster session; the conversation was unsolicited, and I did not review the poster itself beyond the title on display. Standing at the poster, asked why the adolescent drug-resistant population is still excluded, describing families sent away empty-handed until 18. I replied that I am able to discuss only the approved adult indication for XCOPRI, that I could not comment on any use below 18, and that the question would be routed to Medical Information. Closed by requesting whatever adolescent safety or pharmacokinetic data exists, published or pending.

***Conversation — Jaromir Cavanaugh, MD (Granite Bay Epilepsy Center)***

Caught between sessions in the poster hall; the conversation was short and entirely unprompted. Explained that drug-drug interactions on top of already long medication lists in patients over 65. Talked through the pediatric-to-adult transfers in general terms, without any specific case.

***Conversation — Zubin Abernathy, MD (Cedar Ridge Regional Epilepsy Program)***

Introduced by a colleague from the same region and talked for a few minutes. Asked me to send confirmation of whether any adolescent safety data is available yet. Coverage paperwork is handled by one person here, which came up as a general frustration. Talked through the surgical workup list in

general terms, without any specific case. Raised, without any prompting, the wait-until-18 pattern, called an uncomfortable way to practice.

***Conversation — Kiara Vandergriff, MD (Beacon Hill Neurology Associates)***

The conversation happened while waiting for a session room to clear. Closed there — nothing that needed follow-up. The reluctance here is how the medication list in patients over 65 makes the whole decision feel heavier. Referral patterns into this clinic have changed, and the refractory group is a bigger share than it was.

Follow-ups have been logged in the interaction system.